

The Virus ZONE

CARLO URBANI Italian doctor from WHO and president of *Médecins Sans Frontières* (Italy). He is the first person to detect the SARS virus in an American businessman at a French Hospital in Hanoi in March 2003. His early detection helps isolate cases and prevent the infection from spreading further, but he succumbs to the disease a few days later.



ROBERT G. WEBSTER He is the first virologist to find the link between bird flu and human influenza in 1957. He co-authors *The Encyclopedia of Virology* and helps develop flu vaccines at the St Jude Children's Research Hospital in Memphis, where he continues to work. Webster is also Director, World Health Organization Collaborating Centre on the Ecology of Influenza Viruses.



RIYA SHEKH The 14-year-old girl from Pune's Jehangir Hospital becomes the first person to succumb to the H1N1 virus in India in August 2009. Despite being on anti-swine flu drugs, her condition deteriorates and she dies of multiple organ failure. Her death results in widespread panic in the country.



SPIN-OFFS

Born of Illness

Virus Overload

As Indians are introduced to newer strains, the media goes overboard with round-the-clock updates of the spread of the virus and all those affected, from Miley Cyrus to Narendra Modi.

Obesity

Higher incomes, a sedentary lifestyle and improper diets make several young Indians obese, ensuring they are sitting ducks for heart ailments and diabetes.

Medical Tourism

A lot of foreigners visit India as it offers services at par with the West at a lower cost. And as a package deal, the patients are encouraged to pay a visit to the country's tourist spots.

Corporatisation of Hospitals

As India becomes prosperous and public healthcare facilities are found wanting, large-scale corporatisation of hospitals is taking place to meet the growing demand.

country reports one million tobacco-related deaths each year. More than half of adult males in India use tobacco products and recent estimates indicate that smoking will cause 9,30,000 deaths in 2010.

When we juxtapose our global health achievements and failures, we see the gross disparities that exist across and within countries. We live in a world in which life expectancy gaps between countries now reach 50 years, and there are almost as many countries with infant mortality rates greater than 100 per 1,000 live births as there are with rates less than 4 per 1,000 live births. India is situated in the middle of the pack, with an IMR of roughly 30. As another jarring example of health inequality, India has almost 500 times as many births as Norway; but it has 15,500 times as many maternal deaths.

Notwithstanding the deficits, disparities, and new health challenges facing the world, we are poised to make great progress in public health in the coming decade. The most promising development is the fact that global health has moved to the centre of the world's radar screen. We are witnessing a massive infusion of financial resources for dealing with health issues and a growing commitment to strengthening national health systems. Indeed, we have seen a doubling in the amount of funding available for global health, from an estimated \$10.7 billion in 2000 to \$21.8 billion in 2007. This is largely due to major funding efforts such as the Global Fund to Fight AIDS, Tuberculosis, and Malaria; the Gates Foundation; and PEPFAR as well as the US's commitment to combating HIV/AIDS. We have a better un-

derstanding of the role of lifestyle and diet in promoting health and preventing/managing disease. Moreover, research and public health efforts are under way: mapping the human genome, researching new vaccines, and funding global efforts to combat HIV/AIDS, among others.

A key wildcard has to do with the world's willingness and capacity to realign its system of global health governance and cooperation to the new realities so that we can take decisive action to improve global population health. Currently, an overlapping set of national and international institutions take responsibility and fund a confusing array of efforts that leave some countries, and health problems, with inadequate attention. An insufficient number of doctors and health workers, in places where they are most needed, and a proliferation of infectious disease epidemics throw light on the need for international cooperation in dealing with health problems. Public health problems are fundamentally cross-national in nature, either with respect to the spread of diseases or the social, economic, and political ramifications of health issues. Hopefully, the world will recognise during the next decade that cross-national problems demand cross-national solutions and will be able to summon the will to expand and strengthen our mechanisms for global cooperation in the health arena.

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